



Contents lists available at NCBI

The American Journal of Science and Medical Research

Journal homepage: <https://ajsmrjournal.com/>



Research Article

Ethnomedicinal plants and traditional healing practices among Tribal groups of Telangana, India: Diversity, Uses and Conservation Imperatives



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<https://dx.doi.org/10.5281/zenodo.20476308>

Received: 21 April 2026

Revised 25 May 2026

Published on 31 May 2026

ISSN: 2377-6196© 2025 The Authors.

Published by AIRA

Keywords: Ethnomedicinal plants, tribal communities, traditional botanical knowledge, conservation strategies, Telangana.

ABSTRACT

Traditional botanical knowledge (TBK) related to the use of medicinal plants plays an important role in the primary healthcare system of tribal and rural communities. However, rapid socio-economic changes and environmental degradation are leading to the gradual loss of this valuable indigenous knowledge. The present study aims to document the ethnomedicinal plants used by tribal communities of Telangana, India, and to highlight the importance of conserving these valuable biological resources. Field surveys were conducted in selected tribal areas through interviews with traditional healers, elderly villagers, and knowledgeable informants using semi-structured questionnaires and group discussions. The study revealed about 280 plant species belonging to 89 families and 235 genera are traditionally used for the treatment of various ailments such as fever, skin diseases, digestive disorders, respiratory problems, and wounds. Of these, herbs were predominated with 106 species, followed by trees (83 spp.), climbers (57 spp.) and shrubs (34 spp.). Leaves were found to be the most frequently utilized plant part, followed by roots, bark, and fruits. The results also indicate that unsustainable harvesting, habitat destruction, invasive species, and forest fires pose serious threats to the availability of medicinal plants in the region. Documentation of this traditional knowledge is essential for preserving indigenous healthcare practices and for promoting further pharmacological research. The study highlights the need for sustainable utilization and conservation strategies to protect medicinal plant diversity and associated traditional knowledge systems.

1. Introduction

Indigenous communities living in forested regions possess extensive knowledge about the utilization of plants for food, medicine, and other daily needs. This knowledge, which has been developed through long-term interaction with natural ecosystems, is generally transmitted orally from one generation to another. However, modernization, habitat loss, and socio-cultural changes are gradually eroding these traditional knowledge systems. Consequently, systematic documentation of ethnobotanical information has become increasingly important. Traditional knowledge has also gained recognition as a valuable resource for modern scientific research, particularly in the discovery of new pharmaceutical compounds and bioactive substances. In recent years, ethnobotanical research has attracted considerable attention

due to its potential role in biodiversity conservation, sustainable resource management, and the improvement of rural healthcare systems.

Ethnobotanical investigations have led to the documentation of a large number of wild plants used by the indigenous people to meet their multifarious needs. According to the All India Ethnobiological Survey conducted by the Ministry of Environment and Forests, Government of India, local communities across the country utilize more than 8,000 plant species for various purposes. Among these, nearly 1,800 species are used in Ayurveda, about 600 species in Siddha medicine, and approximately 400 species each in Unani and Homeopathy systems of treatment. The Botanical Survey of India (Howrah) was initiated an official programme in the Economic Botany Section and with this, the ethnobotanical

exploration studies were carried out by Janaki Ammal for the first time in 1954. Consequently, Jain started intensive field exploration studies among tribal areas of central India (Jain, 1963 a-c; 1964 a-c; 1965 a,b).

All these publications have triggered the young minds of botanists, anthropologists and ayurvedic medical practitioners in ethnobotanical activities in early sixties. The All India Coordinated Research Project (AICRP) on Ethnobiology came into operation from 1982 at NBRI, Lucknow, and four centres/circles, namely, Shillong, Howrah, Coimbatore and Port Blair of Botanical Survey of India. Mudgal (1987) documented a synoptic account of ethnobotanical studies carried out in India and Binu et al. (1992) reported the ethnobotanical work carried out in India. Later, Lalramnghinglova and Jha (1999) reviewed the work on ethnobotany of the World with special reference to India. An important prerequisite for proper utilization of raw materials of the country is the survey of its natural resources and the preparation of an inventory. It is necessary to have a full knowledge about the occurrence, abundance, distribution and phenology of various plants for their sustainable utilization. The forests of Telangana have great potential in terms of economic benefits to ethnic people. The Telangana State is one of the timber and non-timber rich forests in the country.

In Telangana, almost certainly for the first time, the ethnobotanical uses of few medicinal plants were recorded in the Forest Flora of Hyderabad (Khan, 1953). Kapoor and Kapoor (1980) listed the wealth of medicinal plants from Karimnagar district. Ramarao (1988) visited Rangapur and Thupakulagudem hamlets and interacted with Koyas and Lambadis of Warangal district for his doctoral work. Ravishankar (1990) and Mamidala et al., (2020) worked on the ethnobotany of Adilabad and Karimnagar districts for his doctoral degree. Ravishanker and Henry (1992) published a brief account of Ethnobotany of Gonds of Adilabad district. Later on, notes on ethnomedicinal uses of some important plant taxa of Mahabubnagar district (Pullaiah and Kumar, 1996; Kumar and Pullaiah, 1998) and folk treatment of bone fracture in Ranga Reddy district (Padmarao and Reddy, 1999) were published. Reddy et al. (1998) recorded the ethnoveterinary plants from Warangal district and of these, 49 plant taxa were included in Dictionary of Ethnoveterinary Plants of India by Jain (1999) as an appendix.

Upadhyay and Chauhan (2000) provided an account of the ethnobotany of Gundala mandal in Khammam district. Reddy and Rao (2002) documented the folklore and ethnomedicinal drugs from Ranga Reddy district. Reddy (2002), Kumar et al., (2017); Poojari et al., (2014) documented ethnobotanical plants for Khammam district. Reddy and Raju (2002) published the ethnobotanical uses by Konda reddis of Mothugudem, and Raju and Reddy (2005) enlisted the ethno-botanico-medicinal plants for diarrhoea and dysentery.

A brief report of the ethnoveterinary practices by Koyas in Pakhal wildlife sanctuary by Murthy et al. (2007) whereas Reddy (2008) recorded the medicines from bio-fencing plants from Nalgonda district. Sreeramulu (2008) recorded 313 ethnomedicinal plants from Nalgonda and Warangal districts for his doctoral degree. Reddy et al. (2010) reported 82 medicinal plant species of ethnic use in Medak district. The utilization pattern and diversity of NTFPs from Adilabad district was documented by Omkar et al. (2012). Later, Sreeramulu et al. (2013) reviewed the ethno-botanico-medicine for common human ailments in northern (Warangal) and

southern (Nalgonda) Telangana districts. Suthari et al. (2014a) investigated the traditional knowledge of medicinal plants used by Koya community inhabiting in and around Eturnagaram wildlife sanctuary area and they recorded 237 species of 75 families of Magnoliophyta and four ferns of Pteridophyta as the resources of medicine. Suthari et al. (2014b) enumerated 204 climbing plants of northern Telangana and their ethnomedicinal uses. Saidulu et al. (2015) published the Ethnobotany of Pocharam wildlife sanctuary in the former Medak and Nizamabad districts. Recently, Suthari and Raju (2016) reported the traditional knowledge on 124 flowering plant taxa for poisonous snake-bites used by Koyas of Warangal district whereas Suthari et al. (2016a) documented 470 species of 318 genera and 95 angiosperm families used by Koyas of Warangal North Forest Division of northern Telangana. Mohan et al. (2017a) enlisted 198 ethnomedicinal plants of 165 genera belonging to 72 families whereas Mohan et al. (2017b) recorded 22 antirheumatic plant taxa of 22 genera and 17 families from Kawal wildlife sanctuary of Mancherial district. Suthari et al. (2018) documented the synoptic account on ethnobotanical aspects among the tribal communities in Telangana. Suthari et al. (2021) listed the utility of wild food plants by indigenous tribes from Telangana where Omkar et al. (2022) and Suthari et al. (2023a, b) documented socio-economic empowerment in the rural economy through NTFPs, gum and resin bearing plant taxa and iron deficiency and heredity anaemia in ethnic tribes of Mulugu district of Telangana, respectively.

2. Materials and Methods

2.1. Study area

The present study was conducted in selected forest and tribal-inhabited regions of Telangana State, India. The region lies between latitudes 16°45'-19°55' N and longitudes 77°28'-80°47' E and is characterized by tropical dry deciduous forests. The study area includes tribal settlements located near forest ecosystems where local communities depend heavily on plant resources for healthcare, food, and livelihood. Major tribal groups such as the Koyas and Gonds are largely concentrated in the districts of Bhadrachari Kothagudem, Mulugu, and Adilabad, whereas the Chenchus inhabit the forest regions of the Nallamala hills, particularly within the Amrabad and Nagarjunasagar-Srisailem Tiger Reserve areas. The Lambadis (Banjaras) are more widely distributed across agricultural and rural landscapes.

2.2 Data collection

Field surveys were carried out during different seasons to document ethnomedicinal plant species and their traditional uses. Information was collected through direct interaction with local inhabitants, traditional healers, and elderly members of tribal communities who possess extensive knowledge of medicinal plants. Semi-structured interviews, group discussions, and personal observations were employed to gather ethnobotanical information. Data on local plant names, plant parts used, methods of preparation, mode of administration, and ailments treated were carefully recorded. Plant specimens mentioned by informants were collected from their natural habitats during field visits. The collected specimens were identified and updated using www.worldfloraonline.org. The voucher specimens were prepared and preserved for future reference at herbarium of Centre for Floristics & Ethnobotanical Studies (CFES), Vaagdevi Degree & PG College (A), Warangal (acronym: VHW).

2.3 Data analysis

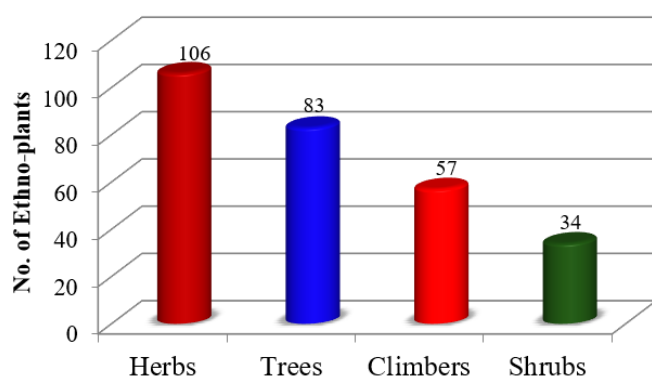
The ethnobotanical data obtained during the study were systematically compiled and analyzed. Medicinal plant species were categorized based on their family, habit, plant parts used, therapeutic applications. The collected information was evaluated to understand the diversity of medicinal plants and their importance in traditional healthcare practices.

The family-wise distribution of plant species recorded in the study area that shows considerable variation in species richness. Among the documented taxa, Fabaceae was the most dominant family with 40 species, contributing significantly to the overall floristic diversity. This was followed by Apocynaceae with 16 species and Malvaceae with 15 species, indicating their notable representation in the vegetation. Euphorbiaceae was represented by 11 species, while Asteraceae accounted for 10 species, showing moderate diversity. In addition to these dominant families, 47 families represented by a single species each. This pattern indicates that while a few families contribute a major share of the species richness, many other families occur with limited representation, reflecting the high taxonomic diversity and heterogeneous nature of the plant community in the study area (Table 1).

Table 2. Life-form composition of the recorded ethnomedicinal plant species from Telangana.

Sl. No.	Habit	No. of Species	%
1	Herbs	106	37.86
2	Trees	83	29.64
3	Climbers	57	20.36
4	Shrubs	34	12.14

Figure-1. Graphical representation of ethnomedicinal plants recorded habit-wise



The analysis of growth forms recorded in the study area reveals noticeable differences in the representation of plant life forms. A total of 280 plant species were documented and categorized into four major habit groups: herbs, trees, climbers, and shrubs. Among these groups, herbs were the most dominant with 106 plant taxa, accounting for the largest share of the total flora. The predominance of herbs associated with their fast growth, short life span, and ability to adapt quickly to seasonal and disturbed environments. Herbaceous plants often occupy open spaces, forest margins, and disturbed habitats, which may explain their higher occurrence in the surveyed area. Trees formed the second largest group with 83 species, indicating a well-represented arboreal component in the vegetation. The

presence of a substantial number of tree species suggests that the area supports a relatively stable ecosystem capable of sustaining perennial woody plants. Trees contribute significantly to ecological balance by providing shade, habitat, and food resources for many organisms while also influencing microclimatic conditions.

Climbers were represented by 57 species, showing moderate diversity within the plant community. Climbers usually depend on trees and shrubs for structural support, and their occurrence reflects the availability of suitable host vegetation. The presence of climbers also enhances structural complexity and species interactions within the ecosystem. In contrast, shrubs were comparatively fewer, with 34 species recorded in the study area. Shrubs generally occupy the intermediate layer of vegetation between herbs and trees. Their relatively lower representation may be influenced by competition with dominant herbaceous species or environmental factors that favor other growth forms (Table 2; Figure 1).

2.4 Conservation Imperatives and Future Prospects

The conservation of medicinal plant resources is essential for maintaining biodiversity as well as sustaining traditional healthcare systems. Increasing pressure on forest ecosystems necessitates the implementation of effective conservation and management strategies to ensure the long-term availability of these valuable plant species.

In-situ Conservation: The establishment of Medicinal Plant Conservation Areas (MPCAs) within forest divisions is crucial. The Amrabad, Pakhal, Eturnagaram and Kawal Tiger Reserves inadvertently serve as in-situ sites for species like *Rauvolfia serpentina*. Establishing protected areas such as wildlife sanctuaries, and community conservation zones helps safeguard natural populations of medicinal plants. Sustainable forest management practices and regulated harvesting can also contribute to maintaining ecological balance and promoting natural regeneration.

Ex-situ Conservation: Initiatives by the Telangana State Medicinal Plants Board (TSMPB) to promote the cultivation of threatened species (like *Ashwagandha*, *Andrographis*, *Aloe*, *Cissus* and *Gymnema*) in farmers' fields can reduce pressure on wild stocks. Botanical gardens, seed banks, tissue culture laboratories, and medicinal plant nurseries play an important role in conserving rare and threatened plant species. Cultivation of medicinal plants in controlled environments can also reduce pressure on wild populations.

Documentation and Intellectual Property: There is an urgent need to digitize the ethnomedicinal knowledge of tribal healers under the People's Biodiversity Registers (PBRs) to prevent biopiracy and ensure benefit-sharing under the Biodiversity Act, 2002.

Sustainable harvesting practices: Adopting non-destructive and scientifically guided harvesting techniques is essential for maintaining plant populations in the wild. Collecting only specific plant parts, harvesting during appropriate seasons, and avoiding uprooting of entire plants can help ensure sustainable utilization of medicinal plant resources.

Community participation: Local and tribal communities play a crucial role in the conservation of forest resources. Encouraging community-based conservation initiatives, awareness

programmes, and participatory forest management can help protect medicinal plants while also supporting the livelihoods of forest-dependent people.

Documentation and knowledge preservation: Systematic documentation of traditional ethnobotanical knowledge is necessary to prevent the loss of valuable indigenous information. Recording traditional medicinal practices and integrating them with scientific research can support the development of new pharmacological products and promote sustainable utilization of plant resources.

Integration with AYUSH: Integrating validated ethnomedicinal practices with the AYUSH system can provide economic incentives to tribal healers and encourage conservation

Research Gap and Objectives of the Present Study

Although several ethnobotanical studies have been conducted in different parts of southern India, systematic documentation of traditional medicinal plant knowledge among many tribal communities of Telangana remains limited. Rapid socio-economic changes, habitat degradation, and declining interest among younger generations threaten the continuity of this indigenous knowledge. Moreover, many medicinal plant species used by tribal healers have not been adequately recorded, scientifically validated, or assessed for their conservation status. Therefore, there is a pressing need to document and preserve ethnomedicinal knowledge before it disappears.

The present study aims to document the traditional knowledge related to medicinal plant use among tribal communities of Telangana. Specifically, the objectives of the study are to: Document the diversity of medicinal plants used by local tribal communities; record traditional methods of preparation and mode of administration of herbal remedies; identify plant species used for the treatment of various human ailments; assess the threats affecting the availability of medicinal plants and highlight the need for conservation strategies. This study is expected to contribute to the preservation of indigenous knowledge, provide baseline information for pharmacological research, and support the sustainable conservation and utilization of medicinal plant resources.

2.5 Threats to the Availability of Medicinal Plants

The availability of medicinal plant resources in forest ecosystems is increasingly threatened by several anthropogenic and ecological factors. These pressures not only reduce the natural populations of valuable plant species but also affect the traditional knowledge systems associated with their use.

Habitat destruction: Rapid conversion of forest areas for agriculture, irrigation projects, infrastructure development, new roads laying and mining has resulted in extensive habitat loss and fragmentation. Such disturbances alter natural ecosystems and significantly reduce the distribution and regeneration potential of many medicinal plant species.

Overexploitation: Several medicinal plants with high commercial demand are harvested unsustainably from the wild. Destructive harvesting methods, such as uprooting entire plants, excessive removal of bark, or collection before seed maturation, adversely affect plant regeneration and lead to the gradual depletion of these species in their natural habitats.

Invasive species: The proliferation of invasive species like *Hyptis suaveolens*, *Lantana camara*, *Chromolaena odorata* and *Parthenium hysterophorus* chokes the growth of native medicinal herbs (Suthari et al., 2016b). These species aggressively colonize degraded and open habitats, outcompeting native vegetation and suppressing the growth of indigenous medicinal plants by monopolizing nutrients, space, and sunlight.

Forest fires: Frequent forest fires, whether natural or human-induced, can severely damage vegetation and reduce the abundance of medicinal plants. Fires not only destroy mature plants but also affect soil fertility, soil pH and texture, seed banks, and the regeneration capacity of many forest species.

Climate change: Changes in temperature, rainfall patterns, and prolonged drought conditions influence plant distribution and phenology. Such climatic variations may affect the growth, flowering, and survival of several medicinal plant species, thereby altering their availability in natural ecosystems.

Loss of traditional knowledge: Traditional ethnomedicinal knowledge is primarily transmitted orally across generations. However, increasing modernization, urban migration, and reduced interest among younger generations are leading to a gradual decline in this indigenous knowledge.

3. Conclusion

The study highlights the rich ethnomedicinal knowledge possessed by tribal groups of Telangana. Telangana represents a significant ethnomedicinal hub in Southern India. The synergy between the state's floral wealth and the indigenous knowledge of its tribal communities offers immense potential for drug discovery and rural healthcare. A considerable diversity of plant species is traditionally used for treating various ailments. However, increasing anthropogenic pressures and intrusion of invasive species threaten the availability of these valuable resources. Therefore, proper documentation, sustainable utilization, and conservation imperatives are essential to preserve both medicinal plant diversity and associated traditional knowledge and the sustainability of this resource is precarious. A balanced approach involving scientific validation, conservation of threatened species, and community participation is essential to preserve the ethnomedicinal heritage of Telangana for future generations.

Competing interests:

The authors declare that they have no competing interests

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*From National Conference on Advances in Life Sciences:
Present & Future (NCALS-2026) | 24-25 March 2026 |
Organized by: Department of Zoology, Kakatiya University,
Warangal-506 009, Telangana State, India*